Northbridge General Hospital (fictional)

100 Example Avenue, Northbridge - fictional address, telephone 000 0000

Cardiology Outpatient Consultation

Department of Cardiology

PATIENT **Meera Iyer**	UHID **MG25002**	AGE / SEX **71 years / Female**
REPORT DATE **08 Oct 2025**	REFERENCE NO. **CON-14003187**	DEPARTMENT **Department of Cardiology**

Reason for Visit

Six-monthly follow-up of paroxysmal atrial fibrillation (AF) and hypertension (HTN). She reports palpitations two to three times a month, each lasting under an hour, without syncope.

History

Mrs. Iyer is a 71-year-old woman with paroxysmal AF diagnosed in March 2024 and HTN for 15 years. **Pertinent negatives:** No history of stroke, TIA or major bleeding. No history of heart failure or valvular disease.

Stroke and bleeding risk: CHA2DS2-VASc score 3 (age 65-74, female sex, hypertension). HAS-BLED score 1 (age over 65).

Examination

- Blood pressure 138/82 mmHg
- Pulse 66 /min, irregularly irregular
- Weight 62 kg
- No peripheral oedema

Tests Reviewed

- ECG: sinus rhythm 68 /min, no ST change
- Echocardiogram (Sep 2025): normal LV size
- Creatinine 0.9 mg/dL (05 Oct 2025), eGFR 63

Current Medications

Medicine	Brand	Dose	Frequency	Indication
Rivaroxaban	Xarelto	20 mg	Once daily with evening meal	Stroke prevention in AF
Bisoprolol	Concor	2.5 mg	Once daily in the morning	Rate control
Amlodipine	Norvasc	10 mg	Once daily	Hypertension
Ramipril	Tritace	5 mg	Once daily	Hypertension

Allergies

Shellfish: urticaria. No known drug allergies.

Impression

Paroxysmal AF with infrequent, well-tolerated episodes on rate control and anticoagulation. Hypertension adequately controlled.

Plan

- Continue anticoagulation and rate control unchanged.
- Annual renal function check; reassess anticoagulant dose if renal function declines.
- Review in 6 months, sooner if palpitations become prolonged.

Electronically signed: **Dr. Samuel Ortega, Consultant Cardiologist** on 08 Oct 2025

*** End of report ***

SYNTHETIC DATA — FOR DEMONSTRATION ONLY